SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Weekly paclitaxel + trastuzumab → trastuzumab continuation

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adjuvant HER2-positive, node-negative breast cancer; tumour ≤3 cm · NCCP 00815 · version 3 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Adjuvant treatment aims to reduce recurrence; confirm the exact treatment setting and sequence with your team.

Schedule
Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Required blood tests and cardiac monitoring discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Numbness / tingling, sometimes lasting
- ☐ Hair loss, muscle or joint pain
- ☐ Infusion reaction
- ☐ Mouth, bowel, skin and nail effects

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction / heart failure
- ☐ Infusion or allergic reaction
- ☐ Diarrhoea and fatigue
- ☐ Rare serious lung effects

Discuss the treatment setting, alternatives, exact schedule, common and serious drug-specific risks, fertility and pregnancy precautions, and the monitoring and urgent-contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00815 v3.

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